جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فوزيه جاد جاد ابو صفيه
المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه اول **العنوان :** دمنهور
رقم البطاقة : 25307111400302 **رقم التليفون :** 01112233516 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 11-07-1953 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فوزيه جاد جاد ابو صفيه
National ID: 25307111400302



تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

Patient Name: Fawzeya Gad Gad Abu Safeya	Date : 23/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 72 Year(s)
Medical ID:	Pat. ID: 41619

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITHOUT CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images.

FINDINGS:

****Reported case of breast cancer:**

****Reviewing the attached report dated on 08-10-2024:**

NCCT CHEST:

- **Still noted,**
  - Right breast diffuse skin thickening
  - Right axillary lymph nodes with mild progression of the left axillary nodes, largest measures about 1 cm compared to 0.7 cm in the previous study
  - Tiny bilateral subpleural pulmonary nodules.
  - Small mediastinal lymph nodes
  - Cardiomegaly is still seen with congested pulmonary vasculature and diffuse ground glass veiling, worrisome for element of pulmonary edema.



- No gross cardiac abnormality.

NCCT Abdomen and Pelvis:

- **Still seen,**
  - Atrophic left kidney showing significantly thinned cortex and cortical calcifications, small cortical cyst and small hyperdense stone.
  - The gall bladder shows average wall thickness. Its cavity shows dense stone measures 1cm.
  - Diffuse atheromatous calcifications of the abdominal aorta and its branches.
  - Small fat containing umbilical hernia.
- **Currently,**

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Fawzeya Gad Gad Abu Safeya	Date : 23/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 72 Year(s)
Medical ID:	Pat. ID: 41619

  - Right renal multiple tiny dense stones.
  - Multiple abdominal subcentimetric enlarged lymph nodes.

- The liver is average in size showing homogenous CT density with no focal lesions or biliary radicles dilatation on non-contrast basis.
- Normal CT appearance of the spleen, pancreas and right kidney.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No free collection.



The scanned skeleton:

- **Still seen,**
  - Few sternal body hypodense lesions, the largest about 1 x 1.5 cm.
  - Small sclerotic focus at L2 vertebral body.
- **Currently seen**, Bilateral sacroiliitis.
- No vertebral structural collapse or extra-osseous soft tissue component
- **Opinion:**

***Reported case of breast cancer running the above described changes since last study.*

Dr.Heba El Balshy.MD

Lecturer of Diagnostic and Interventional Radiology,

NLI-Menoufia Univeristy

CT Scan
[[DOCTYPE:CT_SCAN]]



فتح ملف ثدي جديد
م/ امنا ل سيد



ERM036725092

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National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / فوزيه جاد جاد ابوصفيه	Request No.	: 13008907
Patient No.	: 25307111400302	Request Date	: 16/08/2023
Path No.	: 2692/23	Verification Date	: 29/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 70 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 15/08/2023, 4:45 pm.

Gross Examination:
- 4 grayish white tissue cores up to 2 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive lobular carcinoma.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive lobular carcinoma .
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr.Lobna Omar.
Dr.Menna Nabil.

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / فوزيه جاد جاد ابوصفيه	Request No.	: 13008907
Patient No.	: 25307111400302	Request Date	: 16/08/2023
Path No.	: 2692/23	Verification Date	: 29/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 70 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 16/8/2023 .

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) tained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as ounter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:
IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
PR: Strong nuclear reaction in 40% tumor cells (Allred score 7/8).
Her2:Weak membrane reaction in>10% tumor cells (score +1).
Ki67: 5% nuclear reaction of tumor cells.

Conclusion:
Right breast cancer, Tru-cut biopsy:
Invasive lobular carcinoma, Grade II,
PT:
ER: Positive (score 8/8).
PR: Positive (score 7/8).
Her2: (score +1).
Ki67: Positive in 5% of tumor cells.

Signature

Verified by :
Dr.Heba Hasan

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Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / فوزيه جاد جاد ابوصفيه	Request No.	13008907
Patient No.	25307111400302	Request Date	16/08/2023
Path No.	2692/23	Verification Date	29/08/2023
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	70 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof.Dr,Lubna Omar
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

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Immunohistochemistry
[[DOCTYPE:IHC]]

USTAVE/
OUSSY
INATIONAL

Patient Name	: الأستاذه / فوزيه جاد جاد ابوصفيه	Request No.	: 13001987
Patient No.	: 25307111400302	Request Date	: 13/08/2025
Path No.	: 2525/25	Verification Date	: 16/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 72 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data
Left likely pathologically enlarged lymph node, Right breast cancer on adjuvant chemotherapy .

Nature of specimen
Tru cut biopsy collected 13/8/2025.

Gross specimen
Two grey white tissue cores measured 0.2 cm long , totally submitted

Microscopic examination
Tiny core of fatty &dermal tissue showing adnexae &nerve bundles .No nodal tissue detected.
-No malignancy .

Diagnosis
Axillary lymph node , tru cut biopsy :
Tiny core of fatty &dermal tissue showing adnexae &nerve bundles .No nodal tissue detected.
-No malignancy .

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey .

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY
INATIONAL

Patient Name	: الأستاذه / فوزيه جاد جاد ابوصفيه	Request No.	: 13001987
Patient No.	: 25307111400302	Request Date	: 13/08/2025
Path No.	: 2525/25	Verification Date	: 16/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 72 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Gad Gad Abu Safeya	Date : 08/03/2025
Exam : MAMMOGRAPHY BOTH	Age : 71 Year(s)
Medical ID: 3300/2023	Pat. ID: 41619

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right breast cancer on adjuvant chemotherapy

Technique

- Right MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- Right diffuse skin thickening with underlying diffuse areas of architecture distortion. And this was confirmed with complementary US
- Complementary ultrasound showed no left solid or cystic mass lesions.
- Normal left skin thickness and nipple-areola complex.
- Right axillary ill-defined pathological axillary lymph node seen in level I measuring 17x16 mm (extracapsular invasion)
- Left single pathological axillary lymph node seen at level I showing diffuse cortical thickening reaching 0.35 cm and muffled hilum

Opinion:

- Right locally advanced malignant findings with ipsilateral pathological lymph nodes with extracapsular invasion (BIRADS 6)
- Left indeterminate axillary lymph node seen in level I. for FNAC (BIRADS 4)

Yours truly

Signature Dr. Yasmin Nada

08/03/2025 11:39 AM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Gad Gad Abu Safeya	Date : 08/10/2024
Exam : C.T ABDOMEN &PELVIS	Age : 71 Year(s)
Medical ID: 3300/2023	Pat. ID: 41619

FOLLOWUP NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:

Compared to last study dated 4-9-2023, the current study revealed:

Chest Findings:

- Mild regression of the previously seen right breast diffuse skin thickening
- Stationary course of the right axillary lymph nodes with mild progression of the left axillary nodes, largest measures about 1 cm compared to 0.7 cm in the previous study
- Still noted the small bilateral subpleural pulmonary nodules, ranging 0.5-0.6 cm with no significant changes since last study
- Stationary course of the previously noted small mediastinal lymph nodes
- Cardiomegaly is still seen with congested pulmonary vasculature and diffuse ground glass veiling, worrisome for element of pulmonary edema
- No pleural or pericardial effusion detected.

Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions along the basis of the non enhanced study. No biliary radicles dilatation.
- Still seen the gall bladder radiodense stone measuring about 1 cm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Still seen atrophic left kidney showing significantly thinned cortex and cortical calcifications, small cortical cyst and small hyperdense stone
- Normal size, parenchymal thickness & density of right kidney still showing few tiny calcific foci, likely vascular rather than small stones

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Fawzeya Gad Gad Abu Safeya	Date : 08/10/2024
Exam : C.T ABDOMEN &PELVIS	Age : 71 Year(s)
Medical ID: 3300/2023	Pat. ID: 41619

- Normal non enhanced appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with small about 0.2 cm hyperdense stone resting upon its left posterolateral wall. No filling defects, masses or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Diffuse atheromatous calcifications of the abdominal aorta and its branches
- Still seen small fat containing umbilical hernia.

I. **The scanned skeleton:**

- Still seen few sternal body hypodense lesions, the largest about 1 x 1.5 cm.
- Still seen small sclerotic focus at L2 vertebral body.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- Mild progressive coarse of the left axillary lymph nodes
- Mild regressive course of the right breast skin thickening
- Stationary coarse of rest of CT findings.

Dr. Marina Emad
Dr. Ibrahim Abo El Magd, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Oncology Center مركز الأورام

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital
معهد الناصر
مركز الأورام

NASSER INSTITUTE
ONCOLOGY CENTER
معهد ناصر للأورام

NUCLEAR MEDICINE UNIT
وحدة الطب النووي

Patient Name: Fawzia Gad AboSefein *Date:- 25-9-2023*

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:.

- No definite evidence of hot or cold spots that would account for metastatic bone lesions.
- Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are normally seen.

Conclusion:

No definite evidence of metastatic osseous deposits.

Thanks for reference

Nuclear Medicine Unit

Dr.Huda Fathy , MD

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٣٢٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel.: 22039164 - 22039356 Fax : 22039 236

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Oncology Center مركز الأورام
خطابات أورام

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

Nuclear Medicine Unit

Patient Name: Fawzia Gad Gad *Date: 17/9/2024*

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No active hot spot that could account for osseous metastatic lesions .
- Abnormal heterogenous radiotracer uptake is seen at the spine more advanced at the mid dorsal level , lower lumbar vertebrae ,of degenerative changes .
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any osseous metastatic lesion

Thanks for the referral

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR/ Mohamed Emam

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351 Korniech El Nile _ Aghakhan _ Shoubra _ Cairo Tel. 22039164 22039356 Fax: 22039236

Bone Scan
MDCTYPE:BONE_SCAN

مركز د / محمد عبد المعطى للأشعة
مدير عام الأشعة بوزارة الصحة والسكان
د / محمد ابراهيم عبد المعطى
للأشعة
مركز أشعة ابراء والمنصورة
د / مروة محمد حسن
إستشارى الأشعة التشخيصية
والموجات فوق الصوتية

Mrs.: fawzia Gad Gad
Date : 13/08/23

Digitized Mammography and Complementary Ultrasonography for Both Breasts Revealed

Clinical Summary:

Swollen right breast

Findings:

- ***Right breast***
- Thickening of the right parenchyma and skin with obliteration of the sub cutaneous fatty layer with global asymmetry
- complementary ultrasonography, which depicted streaks of edema creeping in between the parenchyma. With hypoechoic area seen between 5-8 o'clock measures about 6x3cm
- At axillary tail shows sopculted lesion seen measures 15x16mm
- Non specific axillary LNs are seen.
- ***Left breast***
- Fairly dense nodular mammary parenchyma; ACR b
- No dominant mass lesion identified
- BY U/S Normal breast parenchyma, No evidence of any solid or cystic lesion.
- No definite signs of malignancy
- Normal nipple/areolar complex.
- non specific axillary nodes are s

Impression:

Right breas(BIRADS 4c), for Clinical and radiological findings are those of an inflammatory process with hyperechoic area in the right breast associated with lymphatic obstruction, for biopsy. The possibility of inflammatory carcinoma is to be considered.
Left breast (BIRADS1)

Much obliged
Dr Marwa M.Hassan

العنوان : شبرا الخيمة - أمام كوبرى عرابى مباشرة - السلام سيتى مول - الدور الرابع
ت : ٠١٠٠٠٧٢١٥٦٦ - ٠١٠٠١٤٥٦٤١٢ - ٠١٢٠٠٧١٠٠٩٩ - ٠١٢٠٠٧١٠٠٣٥
ش ١٠ أمام صيدلية د.أشرف - شبرا الخيمة ٤٦٠١٤٢٦٧ - تسليم الأشعة في الحال

Immunohistochemistry
[[DOCTYPE:IHC]]